



The \$35 Cap and the People It Misses

Student copy (project or hand out)

1 Thirty-five dollars is the number that did the work. Under the Inflation Reduction Act, the Centers for Medicare and Medicaid Services capped out-of-pocket insulin at \$35 a month for Medicare Part D beginning January 1, 2023, and for Part B that July. About 1.5 million Medicare beneficiaries who use insulin stood to benefit, with annual savings the federal government put at \$734 million in Part D and \$27 million in Part B. The figure is clean, the relief is real, and it stops where Medicare stops.

2 The cap reaches Part D and Part B beneficiaries. It does not reach the uninsured. It does not reach most people on private plans, and it does not reach those covered by Medicaid alone. Twenty-nine states and the District of Columbia have written their own \$35 ceilings into state-regulated health plans, but those, too, hold only inside their own borders and their own program rules. The median monthly out-of-pocket price runs about \$40 for someone with private insurance and about \$100 for someone with none.

3 The list prices have come down a long way. Eli Lilly cut several of its insulins by more than 70 percent in 2023; Novo Nordisk followed with reductions of 75 percent on Fiasp and 72 percent on Tresiba, effective January 1, 2026. New suppliers arrived alongside them. Civica Rx launched a long-acting glargine biosimilar this January at a \$45 wholesale price and a recommended ceiling of \$55 for a box of five pens, and California's CalRx put a state-branded glargine on shelves at \$11 a pen. Civica is a nonprofit; CalRx is a state. The companies that already controlled the market did not build either one.

4 That market is narrow. Novo Nordisk, Eli Lilly, and Sanofi together hold more than 90 percent of the global insulin supply and are the only firms selling insulin to patients in the United States. The barriers to a fourth entrant are documented and dull: an interchangeability approval that no insulin product has ever received, trade-secret protection on the manufacturing, patents on the delivery devices, and development costs running \$100 million to \$250 million over seven or eight years. Biosimilars like Semglee and Rezvoglar now sell for 65 to 85 percent less than the brand analogs, which tells you how much room the old prices held.

5 So the headline numbers and the survey numbers do not agree. In October 2025 the American Diabetes Association found insulin still unaffordable for almost 30 percent of people with diabetes. One in six insulin users rationed the drug because of cost; among uninsured respondents, 39 percent said they had rationed in the past six months. Rationing means taking less than the prescribed dose, or skipping it, to make a vial last. Manufacturer assistance programs offer free insulin to some who earn up to 400 percent of the federal poverty level, roughly \$60,240 for an individual in 2026, though the enrollment is restrictive and hard to navigate.

6 In March 2026, senators Warnock, Shaheen, Kennedy, and Collins introduced the INSULIN Act, which would extend the \$35 monthly cap to people with private insurance and set up a pilot grant program for the uninsured. It has not passed. For now the \$35 figure means one thing on Medicare, something narrower in 29 states, and nothing at all for the person paying cash at the counter.

AP-style multiple choice:

1. In the second paragraph ("The cap reaches Part D and Part B beneficiaries..."), the repeated construction "It does not reach" primarily serves to
 - A) concede that the cap has failed and should be repealed by Congress
 - B) enumerate the groups the cap excludes so the limitation registers through accumulation rather than assertion
 - C) prove that state-level caps are more effective than the federal one
 - D) build emotional outrage at the manufacturers who set insulin prices
2. Which of the following best characterizes the writer's overall position on the 2026 insulin price news?
 - A) Falling list prices have effectively solved insulin affordability in the United States
 - B) The \$35 cap is a failure that has helped almost no one
 - C) The three major manufacturers deserve credit for the recent drop in prices
 - D) The documented relief is genuine but partial, leaving large groups still priced out

Discussion questions:

1. The piece reports both a real \$35 cap with documented savings and an ADA survey showing nearly 30 percent still cannot afford insulin. How does placing these facts next to each other, without the writer naming a contradiction, shape what you conclude?
2. The author attributes almost every claim to a named body, figure, or date (CMS, the ADA, specific senators, exact percentages). What does that habit of attribution do to the writer's credibility, and how would the paragraph on rationing read if the numbers were dropped?

Writing prompt (Homework or extended in-class, Q3 argument):

The opener documents a \$35 insulin cap that delivered real, measurable savings to the Medicare beneficiaries it covers, while leaving the uninsured, most privately insured, and Medicaid-only patients outside its reach. Some hold that a targeted policy delivering proven relief to one large group is a legitimate and worthwhile step even when it does not reach everyone; others hold that a benefit structured to miss the people in the greatest need, the uninsured who ration most, is a failure of design rather than a partial success. Write an essay in which you defend, challenge, or qualify the claim that a price cap is worth enacting even when it covers only some of the people it could. Use specific evidence from the opener and your own knowledge or experience to support your line of reasoning.